

Acute Coronary Syndrome — Clinical One-Pager

Early Tx (all ACS) · revascularization timing · discharge meds. Core per 2014/2013 ACC/AHA, updated 2025 ACC/AHA. Adapted from TeachIM.org (CC BY-NC 4.0). Educational use only.

1 · Early Tx — same for STEMI / NSTEMI / UA

Agent	Dose / rule	Avoid
Aspirin	325 mg chewed → 81 qd	major bleed, allergy
Anticoag	heparin if PCI planned; enoxaparin if med mgmt (ESSENCE)	HIT; enox if CrCl<30
NTG	SL 0.4 q5min x3 → IV gtt	RV/inf MI, SBP<90, AS, HCM, PDE5i<24h
Beta-blocker	PO, initiate <24h; mortality benefit	shock, HR<45, AVB, SBP<100
Morphine	last-line, after nitrates+BB	masks angina, ↑mortality
GP IIb/IIIa	not routine; high thrombus burden	bleeding

O2 only if SpO2 <90% — routine O2 in normoxic STEMI may harm (AVOID 2015).

2 · STEMI — time is myocardium

- **PCI-capable:** primary PCI **<90 min** (onset <12h).
- **Non-PCI hospital:** transfer if door-to-balloon **<120 min**; else **fibrinolytics <90 min** → transfer for angio 3–24h.
- PCI > lytics (lytics fail ~35%, more death/MI/CVA/bleed).
- Onset >12h: still PCI unless occluded infarct + no ischemia.

3 · NSTEMI — risk-stratify then time the cath

Tier	Trigger	Timing
Very high	shock, instability, refractory angina, VT/VF	<2 h
High	GRACE>140, TIMI 6–7, new ST dep, ↑trop, age>75	<24 h
Mod–low	GRACE 109–140, TIMI 3–5	25–72 h
Very low	GRACE<109, TIMI 0–1	ischemia-guided

GRACE = in-hosp/6-mo death; TIMI = 14-day death/MI/urgent revasc.

4 · Late / discharge meds

Agent	When · indication
High-intensity statin	<24h; all ACS (atorva 80 / rosuva 20–40)
P2Y12 inhibitor	post-PCI, ~12 mo DAPT; hold 5d pre-CABG
ACEi/ARB	before d/c; all STEMI; NSTEMI if EF<40, HTN, DM, CKD
MRA	before d/c; EF<40, HF, or DM & tolerating BB+ACEi

ACEi/ARB & MRA each carry 30-day mortality benefit.

2025 ACC/AHA ACS update

- **P2Y12:** ticagrelor OR prasugrel preferred over clopidogrel (PCI); DAPT ≥12 mo if not high bleed risk; ticagrelor monotherapy ≥1 mo an option.
- **Lipids:** high-intensity statin ± ezetimibe for all; add non-statin (ezetimibe/PCSK9i/inclisiran/bempedoic acid) if LDL-C ≥70 on max statin.
- **Procedure:** radial > femoral; intravascular imaging for complex lesions; **complete revascularization** (culprit-only in shock).
- Transfusion to Hgb 10 reasonable if anemic & not bleeding; lipid panel 4–8 wk; cardiac rehab.

Do / Avoid & red flags

DO: ASA + anticoag + statin early; risk-stratify NSTEMI; radial access; complete revasc; rehab referral.

AVOID: routine O2 if SpO2≥90; morphine before max nitrates/BB; nitrates/BB in hypotension or RV infarct; delaying STEMI reperfusion; routine non-IRA PCI in cardiogenic shock.

RED FLAGS → immediate cath (<2h): cardiogenic shock, hemodynamic/electrical instability, refractory angina.

Ongoing pain on max therapy = ongoing ischemia → revascularize.